

MUSLIMHACKS 2026 · ELDERLY CARE

أمانة

Amanah

A trust placed in your hands.

Amanah Care. A private record for the family that looks after a parent.

"My Lord, have mercy upon them as they brought me up when I was small."

QUR'AN 17:24

Every one of us will carry this trust. Most of us will carry it with our brothers and sisters, on our phones, between a job and children of our own.

Ammi is 78.

Fatima · 07:52

did she take the morning ones?

Sister A · 08:10

yes. she ate half. who has her this afternoon?

Abdullah · 08:41

I thought Fatima. I have the pharmacy at 11 I think?

Fatima · 09:15

no that was last week. what did Dr Rahman say wednesday??

Sister A · 09:20

scroll up

Three children. Three phones. One group chat.
Nobody knows what she took this morning.
Nobody knows who has her this afternoon.
And a company in California keeps her health history.

1 in 4

American Muslims already care for an older adult. Twice the rate of the general public.

ISPU, Support and Sacrifice, 2026

1 in 4

family caregivers say they cannot coordinate the care. Up from 1 in 5 five years earlier.

AARP / NAC, Caregiving in the U.S., 2020

Not a niche. This is us at forty, with children at home. 52% of Muslims aged 30 to 49 are doing it right now.

So we gave her a record.

Plan

Her day, written once. Morning meds at 8. Dhuhr. The walk after Asr. Night meds at 9, Fatima. Every day becomes a checklist.

Log

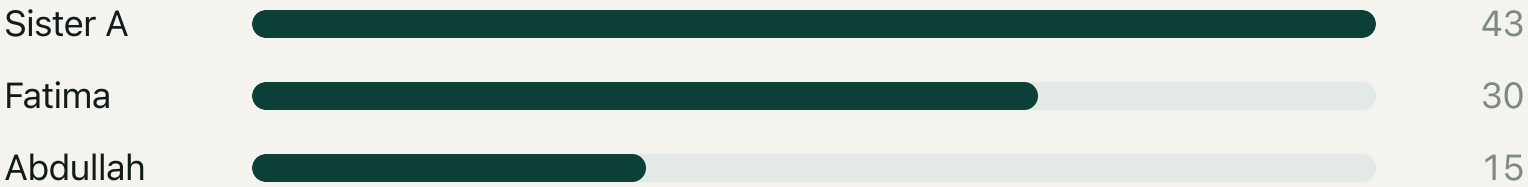
Two taps. Ate half. Meds given. Tired. Everyone in the family sees it the second it happens, on their own phone.

Hand off

When you leave, the card writes itself: what happened, what is next. The next person knows. Nothing falls through.

And the record shows who carried the week, so nobody carries it alone.

Recommendations: rules over the record, on the phone, never a diagnosis.



WHEN THE AMBULANCE COMES

The first ten minutes are questions the family cannot answer.

What did she take, and when. When did she stop eating. When did the confusion start. Her allergies. Her doctor.

One tap

Fourteen days on one page. Medication by day, meals, mood, appointments, contacts. Printed from the phone. Nothing inferred.

Emergency

Every phone in the family lights up, and the sheet opens. Ammi has her own button: I need help.

She is not the user. She is the reason.

Her own screen, in big type. Who is with her today.
What is coming. Her prayers. Who can read her record.

Her preferences ride on every handoff: Urdu, soft halal food, a woman for personal care. Not a setting. Her wishes.

One button. I need help.

SATURDAY, SEPTEMBER 6

Assalamu alaykum, Ammi

Fatima is looking after you today. **Abdullah** is coming next.

Night meds

21:00 · Fatima

Isha

21:15

Fajr ✓

Dhuhr ✓

Asr ✓

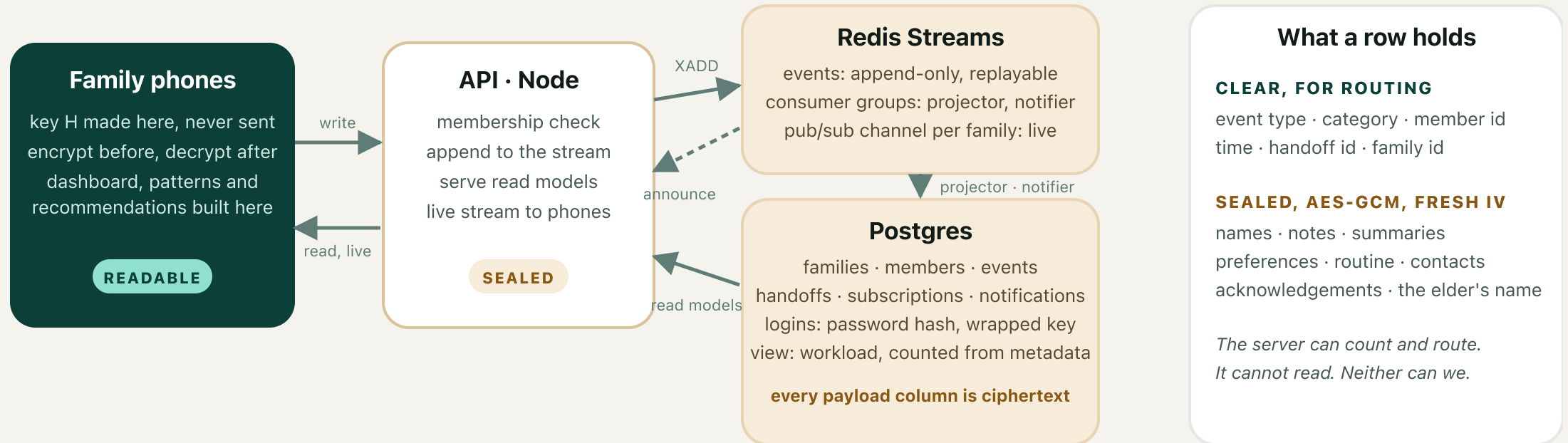
Maghrib

I need help

Who can read your record: Sister A, Abdullah, Fatima.
Nobody else.

HOW IT WORKS

One stream, one database, and a key that never leaves the phone.



Every action is an event. Dashboards, alerts, the hospital sheet and the recommendations are projections over one log. A test proves a written note appears in no row, no stream entry, no response. 114 tests, 99.9% line coverage. Docker Compose, six containers, one command.

The service, and who pays for it.

WHAT IS DELIVERED

To the family: the plan, the log, the handoff that writes itself, the hospital sheet, recommendations. Private by construction.

To a home-care agency: a handoff log for every visit, scoped access for its workers, families who stop calling to ask what happened.

To the elder: her own screen, her wishes on every card, and the truth that nobody outside her family can read her record.

Quebec alone: about **85** social-economy home-care enterprises, **110,000** clients, **8 million** hours a year, **10,000** workers. Réseau de coopération des EÉSAD, 2024. One client in a hundred on an agency plan pays for the team.

WHO PAYS, AND WHY IT HOLDS

Families: free, always. The record must never be a paywall between siblings.

Home-care agencies and support services: per elder per month, for the handoff log, worker access and visit proof. Illustrative: \$30.

Community organisations: sponsor the families they already serve. A welfare committee carries fifty families for less than one event.

Public home care, later: fewer coordination failures are worth paying for. A year-three conversation.

Sound because the payer's gain is operational and measurable, the family's privacy does not depend on the payer, and there is no readable data to sell. Running cost is cents per family; the real cost is people: support, and a nurse reviewing every recommendation.

WHAT COMES NEXT

Now	Everything you saw. Six containers, 114 tests, one family in the room.
Month 1	Five Montreal families, one month. Urdu and Arabic on Ammi's screen. Key recovery between two relatives. Push notifications.
Month 3	One home-care agency pilot with a scoped key for its workers and visit proof. A geriatric nurse reviews the recommendation table. Condition packs as content: dementia first.
Month 6	Native apps, offline first. Voice to structured log, on the phone, opt in. Night-activity sensor with her consent.
Year 1	A thousand families, ten agencies, four languages. The recommendation table opened to community review. A first conversation with public home care.

Care was already happening. Now it has a record. And only the family can read it.

We ask for introductions to community care programs, five families, and a designer for her screen.